

August 12, 2026

Cigna Healthcare
Cigna Appeals Unit
P.O. Box 188011
Chattanooga, TN 37422

RE: Formal Appeal of Claim Denial — Lakendra Roberts (MRN MUSC4738973), Claim MUSC-20250710-4D58-1

Patient	Lakendra Roberts (DOB 1962-07-26)
MRN	MUSC4738973
Member ID / Group	CIG406449781 / GRP-50704
Claim number	MUSC-20250710-4D58-1
Date of service	2025-07-10
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$429.02
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-02-19

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this first-level appeal on behalf of patient Lakendra Roberts (MRN: MUSC8650384, Member ID: CIG406449781, Group: GRP-50704) regarding the denial of Claim MUSC-20250710-4D58-1 for services rendered on July 10, 2025, by Elena V. Castellanos, MD (NPI 1881726354). Cigna Healthcare denied the claim in full — \$429.02 billed — under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the payer remark stating that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the contracted allowed amount of \$319.30.

CLINICAL BACKGROUND

Ms. Roberts is a 64-year-old female with a well-documented, longstanding history of complex chronic conditions managed at MUSC Health. Her active problem list includes chronic kidney disease, diabetic renal disease, hyperglycemia, prediabetes, metabolic syndrome, and anemia — conditions that have been continuously managed since at least 2001. She is maintained on active medications including long-term insulin therapy (Humulin 70/30, initiated 2003), extended-release metformin (initiated 2001), and hydrochlorothiazide (initiated 1995), reflecting the chronic and medically intensive nature of her ongoing care. Prior laboratory findings document a markedly elevated microalbumin-to-creatinine ratio and a significantly reduced estimated glomerular filtration rate, consistent with progressive renal involvement in the setting of her diabetic and metabolic conditions. The July 10, 2025 encounter was billed as CPT 99214 — an established outpatient visit of moderate complexity — with a primary diagnosis of J06.9 (acute upper respiratory infection, unspecified) and a secondary diagnosis of R69, which in this patient's record maps to her active conditions of hyperglycemia and metabolic syndrome.

BASIS FOR APPEAL

The denial rests on the assertion that the visit was not medically necessary for the diagnosis reported. This conclusion is not supported by the clinical context. Ms. Roberts is an established patient with multiple active chronic conditions, including insulin-dependent diabetic renal disease and chronic kidney disease, that require ongoing monitoring and management. An acute upper respiratory illness in a patient of this complexity warrants evaluation at the level of moderate medical decision-making, as intercurrent illness in patients with compromised renal function and glucose dysregulation carries meaningful clinical risk. The selection of CPT 99214 is consistent with CPT coding guidelines for established outpatient visits requiring moderate complexity decision-making, which encompasses the assessment of an acute problem against the backdrop of multiple chronic conditions with potential for complications. The secondary diagnosis code R69 further reflects the active comorbid conditions present at the encounter. Cigna's reliance on an LCD to deny this service does not account for the totality of the patient's clinical picture, and the denial appears to evaluate the primary diagnosis in isolation rather than in the context of the patient's established, medically complex profile.

REQUESTED ACTION

MUSC Health requests that Cigna Healthcare conduct a full clinical review of this appeal, including the complete medical record for the July 10, 2025 encounter and Ms. Roberts's longitudinal problem and medication history. Upon review, we request that the denial be reversed in full and that payment be issued at the contracted allowed amount of \$319.30. Should Cigna require additional clinical documentation or a peer-to-peer review with Dr. Castellanos, we welcome that opportunity and ask that it be arranged promptly given the appeal deadline of February 19, 2026. This appeal is being submitted through the designated channel in accordance with Cigna's appeal process requirements.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record